Bimanual Pelvic Examination — OSCE Checklist

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Prepare and consent → Position → Inspect → Internal palpation → Bimanual → Close**. *Indicated for genitourinary symptoms and in pregnancy —* **not** *as routine screening.*

1. Before the patient is on the couch

☐	Why a pelvic examination is needed; what it involves (speculum, CST, swabs, bimanual) and the position
☐	Past experience of pelvic examinations
☐	Obstetric history: children? vaginal births? (guides speculum size)
☐	Menstrual history: LMP; possibility of pregnancy
☐	Verbal consent; "Just say stop and I will"
☐	Offer a chaperone (advocate for patient and doctor; often a nurse who assists)
☐	Offer to empty bladder first

2. Procedure

☐	Gather equipment (gloves, lubricant, light, bluey, tissues); hand hygiene; gloves
☐	Patient undresses from waist down behind curtain, covers with blanket; enter with permission
☐	Position: supine, knees drawn up, heels together, knees fall apart
☐	Expose waist down; inspect external genitalia
☐	Warn patient; gently touch the introitus, then slowly insert 2 lubricated gloved fingers
☐	Palpate cervix: position, consistency, os, tenderness †
☐	**Cervical excitation**: gently move the cervix side to side — watch the face for pain
☐	Uterus: other hand on lower abdomen pressing down towards internal fingers — size, shape, position (anteverted / retroverted), mobility, tenderness †
☐	Adnexa: internal fingers in each lateral fornix, abdominal hand over iliac fossa — masses, tenderness; both sides
☐	Withdraw fingers; check glove for blood or discharge
☐	Cover patient; leave tissues and bin; privacy to dress



Position: heels together, knees fall apart

3. Closing

☐	Explain findings and next steps; invite questions
☐	Document consent, chaperone, findings, investigations, treatment and follow-up

4. Quick reference †

Finding	Think of
Cervical excitation (motion tenderness)	Pelvic inflammatory disease, ectopic pregnancy
Enlarged, irregular, firm uterus	Fibroids; pregnancy (smooth, soft)
Bulky, tender uterus	Adenomyosis
Fixed, retroverted, tender uterus ± nodules	Endometriosis
Adnexal mass	Ovarian cyst or tumour, ectopic, tubo-ovarian abscess
Blood on glove	Menses, IMB/PMB — correlate with history

- Never uncover the patient yourself if avoidable — ask them to do so when comfortable.
- Pitfalls: no chaperone offer, not warning before touching, examining without watching the patient's face, not checking the glove.